Final new medical topics

Clinical and radiologic image atlas for physicians and medical students

Recommend 493 distinct additions: 128 first-priority topics and 365 next-priority topics. The list consolidates repeated proposals and assigns each topic one home specialty.

First priority covers core teaching diagnoses, common visual differentials, and important emergencies. Next priority extends specialty coverage with less common but useful clinical or imaging appearances. Both groups meet the editorial scope; priority reflects release order.

Selection criteria

- Include distinct diseases, syndromes, injuries, and structural diagnoses with useful patient photographs, endoscopic or fundus images, dermoscopy, or radiologic manifestations. Ultrasound and echocardiography qualify.
- Use pathology as supporting material. A topic whose useful image is only histology, organism microscopy, a laboratory result, or a genetic finding is deferred.
- A teaching appearance need not be unique to one disease. Captions must state the relevant clinical context and avoid calling supportive findings diagnostic by themselves.
- Keep repeated sites, causes, and overlapping phenotypes together when one page can teach them clearly. Record substantive additions to existing pages separately from new pages.

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Addition counts by specialty

Home specialty	First	Next	Total
Breast	1	11	12
Cardiology	14	24	38
Endocrinology	2	13	15
Gastroenterology	25	19	44
Hematology	3	12	15
Immunology	0	14	14
Nephrology	6	12	18
Neurology	14	39	53
Orthopedics	15	77	92
Otolaryngology	18	20	38
Pediatrics	11	59	70
Pulmonology	10	29	39
Rheumatology	1	9	10
Urology	8	27	35
Total	**128**	**365**	**493**

Topics to add

Each numbered entry is one new topic. The brief image anchor explains its clinical or radiologic teaching value. Related entities included within a shared topic are labeled in the anchor or decision audit.

Breast

First priority 1 topic

001 Silicone Breast Implant Rupture. MRI linguine sign of collapsed implant shell; US snowstorm from extracapsular silicone.

Next priority 11 topics

002 Atypical Vascular Lesion of the Irradiated Breast. Small pink/brown or erythematous papules/vesicles arising in irradiated breast skin.

003 Breast Implant Capsular Contracture. Firm, high, spherical, distorted breast after implant placement; Baker III visible distortion, IV pain/hardness.

004 Congenital Amastia and Amazia. Congenital absence of breast mound (amastia) or gland with preserved nipple-areola (amazia); pectoralis present.

005 Granular Cell Tumor of the Breast. Firm superficial mass with skin/nipple retraction; spiculated mammographic mass and posterior shadowing on US.

006 Hyperkeratosis of the Nipple and Areola. Verrucous brown hyperkeratosis involving nipple and areola, unilateral or bilateral.

007 Juvenile Papillomatosis. Adolescent/young adult with localized firm mass; ultrasound clustered cysts of varying size (“Swiss cheese”).

008 Lactating Adenoma. Pregnancy/lactation-associated mobile mass; US circumscribed oval lesion, becoming complex/cystic if infarcted.

009 Pseudoangiomatous Stromal Hyperplasia. Rapidly enlarging mobile mass; mammographic oval mass/developing asymmetry and parallel hypoechoic US mass.

010 Radiation-Associated Angiosarcoma of the Breast. Years after breast irradiation, violaceous bruise-like plaques/nodules in field; skin thickening or enhancing cutaneous mass.

011 Tuberculous Mastitis. Chronic breast mass with skin sinus; US shows heterogeneous collection and fistulous tract, sometimes necrotic axillary nodes.

012 Tuberous Breast Deformity. Constricted breast base, high fold, enlarged areola, and glandular herniation producing a tubular/Snoopy contour.

Cardiology

First priority 14 topics

013 Acute Myocarditis. CMR shows myocardial edema with subepicardial or mid-wall nonischemic enhancement, often lateral, not following a coronary territory.

014 Anomalous Aortic Origin of a Coronary Artery. Coronary CTA shows an anomalous ostium from the opposite sinus, slit-like takeoff, and interarterial/intramural course.

015 Aortic Stenosis. Restricted aortic leaflet opening with Doppler flow acceleration; include calcific adult stenosis and congenital valve morphology as clearly labeled forms.

016 Atrial Septal Defect. Interatrial communication with Doppler shunting and right sided volume loading; show secundum and sinus venosus anatomy and label associated anomalous pulmonary veins.

017 Blunt Traumatic Aortic Injury. CTA shows isthmic intimal injury, pseudoaneurysm or contour defect with periaortic/mediastinal hematoma after deceleration trauma.

018 Degenerative Mitral Valve Disease. Systolic billowing or flail mitral scallop above the annular plane with eccentric regurgitant jet on 2D/3D echo.

019 Fibromuscular dysplasia. CTA/MRA or angiography shows multifocal string-of-beads stenoses, especially in renal or cervical arteries.

020 Patent Ductus Arteriosus. Ductal color jet from descending aorta to left pulmonary artery; large PDA produces left-heart enlargement and pulmonary plethora.

021 Penetrating Atherosclerotic Ulcer. CTA shows a focal contrast-filled crater penetrating aortic plaque, often with adjacent intramural hematoma.

022 Spontaneous Coronary Artery Dissection. Angiographic intimal flap/double lumen or long smooth narrowing from intramural hematoma in a coronary artery.

023 Superior Vena Cava Syndrome. Contrast CT/venography shows SVC narrowing or occlusion and azygos/chest-wall collateral pathways; facial plethora is visible clinically.

024 Takotsubo Cardiomyopathy. Circumferential apical or midventricular ballooning with basal hyperkinesis on echo or ventriculography.

025 Valvar Pulmonary Stenosis. Doming pulmonary valve with turbulent systolic flow and poststenotic main pulmonary artery dilation.

026 Ventricular Septal Defect. Color Doppler left-to-right jet across the ventricular septum; a large shunt enlarges the left atrium and ventricle.

Next priority 24 topics

027 Arterial Thoracic Outlet Syndrome. CTA with arm elevation shows subclavian artery compression, often poststenotic aneurysm and distal emboli; radiograph may show cervical rib.

028 Congenitally Corrected Transposition of the Great Arteries. Echo/CMR demonstrates both atrioventricular and ventriculoarterial discordance; anterior-left aorta and systemic morphologic RV.

029 Cor Triatriatum. Left atrial membrane separates pulmonary venous chamber from distal atrium; appendage lies distal to the membrane.

030 Coronary Artery Fistula. Dilated tortuous coronary artery traced on CTA/angiography to a cardiac chamber or pulmonary artery, with Doppler runoff.

031 Cystic Adventitial Disease. Ultrasound/MRI shows nonenhancing mucin cysts in popliteal arterial adventitia causing smooth eccentric/scimitar narrowing.

032 Discrete Subaortic Stenosis. Echo shows a discrete fibrous/fibromuscular shelf below an normally opening aortic valve with LVOT jet.

033 Double Outlet Right Ventricle. Echo/CMR shows both great arteries arising predominantly from the right ventricle and defines VSD-to-outlet relationship.

034 Double-Chambered Right Ventricle. Echo/CMR shows anomalous muscle bundle dividing RV into high-pressure inlet and low-pressure outlet, with normal pulmonary valve.

035 Generalized Arterial Calcification of Infancy. Plain films/CT show extensive arterial-wall calcification in an infant, often involving aorta and coronary arteries.

036 Heterotaxy syndrome. Cross-sectional imaging shows abnormal organ situs, systemic/pulmonary venous return, and cardiac position/segmental anatomy.

037 Hypothenar Hammer Syndrome. Angiography shows ulnar artery aneurysm/occlusion over the hook of hamate with distal digital emboli.

038 Interrupted Aortic Arch. CTA/3D reconstruction shows complete interruption of the arch with descending aorta supplied through the duct.

039 Middle aortic syndrome. CTA/MRA shows long mid-thoracic/abdominal aortic narrowing with renal/visceral branch stenoses and collaterals.

040 Multisystem Inflammatory Syndrome in Children. Echo shows transient ventricular dysfunction with possible coronary dilation/aneurysm and pericardial effusion after postinfectious illness.

041 Mycotic Aortic Aneurysm. CTA shows rapidly enlarging saccular aneurysm with periaortic inflammatory stranding/fluid or gas.

042 Nonbacterial Thrombotic Endocarditis. TEE shows broad-based small valve vegetations without leaflet destruction or abscess, in a patient with systemic emboli.

043 Papillary Fibroelastoma. Small independently mobile, stalked valvular mass with a fronded/shimmering edge on echocardiography.

044 Partial Anomalous Pulmonary Venous Return. CT/CMR maps anomalous pulmonary veins draining to systemic venous structures; sinus venosus defects may coexist while the fossa ovalis remains intact.

045 Popliteal Artery Entrapment Syndrome. Dynamic duplex/MRA shows popliteal artery compression with plantar flexion and abnormal gastrocnemius relationship.

046 Pulmonary artery sling. CTA shows left pulmonary artery arising from right pulmonary artery and passing between trachea and esophagus.

047 Pulmonary Atresia with Ventricular Septal Defect. CTA maps atretic RV outflow, VSD/overriding aorta, and major aortopulmonary collateral arteries supplying lung segments.

048 Right Aortic Arch with Aberrant Left Subclavian Artery. CTA shows right arch and aberrant left subclavian from Kommerell diverticulum; esophagram may show posterior indentation.

049 Shone Complex. Echo depicts multilevel left-sided obstruction: parachute mitral valve/ring, subaortic ridge, and coarctation.

050 Sinus of Valsalva Aneurysm. A windsock sac arising from a sinus of Valsalva into a chamber, with a jet if ruptured.

Endocrinology

First priority 2 topics

051 Insulinoma. Hypervascular pancreatic lesion on arterial CT/MRI or receptor PET in fasting hypoglycemia.

052 Pituitary apoplexy. Sudden headache/ophthalmoplegia; MRI shows hemorrhagic or infarcted sellar mass.

Next priority 13 topics

053 Aromatase deficiency. Maternal virilization, virilized 46,XX infant, and later multicystic ovaries.

054 Chanarin-Dorfman syndrome. Congenital ichthyosis with Jordan anomaly (lipid vacuoles) in leukocytes.

055 Hereditary vitamin D-resistant rickets. Rachitic metaphyseal cupping/fraying with total alopecia.

056 Juvenile Paget disease. Childhood generalized bone expansion and long-bone deformity.

057 Kenny-Caffey Syndrome. Kenny-Caffey: deep-set eyes, cortical thickening, and medullary stenosis on long-bone films.

058 Nelson syndrome. Generalized hyperpigmentation after bilateral adrenalectomy with enlarging pituitary macroadenoma on MRI.

059 P450 oxidoreductase deficiency. Craniosynostosis, bowed femora, atypical genitalia with adrenal steroidogenic failure.

060 POMC deficiency. Early obesity, adrenal insufficiency, and characteristic red hair.

061 Rathke cleft cyst. Midline nonenhancing sellar cyst with intracystic T2-dark waxy nodule.

062 Sanjad-Sakati Syndrome. Severe growth deficiency and characteristic face with deep set eyes, beaked nose, micrognathia, and congenital hypoparathyroidism; skeletal findings vary.

063 SHORT syndrome. Partial lipodystrophy, Rieger anomaly, and characteristic triangular facies.

064 Van Wyk-Grumbach syndrome. Precocious puberty with delayed bone age, multicystic ovaries, and pituitary hyperplasia in severe hypothyroidism.

065 VIPoma. Pancreatic tail mass and liver lesions with secretory diarrhea and hypokalemia.

Gastroenterology

First priority 25 topics

066 Ampullary Adenocarcinoma. Endoscopic ulcerated or polypoid ampulla; CT/MRCP may show bile and pancreatic duct dilation ending at the ampulla.

067 Annular Pancreas. Pancreatic tissue encircling the second duodenum with proximal gastric/duodenal dilation on CT or upper GI contrast study.

068 Appendiceal Mucocele. Fluid-filled, dilated tubular appendix with mural calcification or layered “onion-skin” appearance on ultrasound.

069 Chronic Mesenteric Ischemia. CTA shows severe ostial mesenteric arterial stenosis/occlusion with collateral vessels; chronic cases often have multivessel disease.

070 Emphysematous Gastritis. CT shows linear or bubbly gas within thickened gastric wall, possibly portal venous gas, with severe systemic illness.

071 Epiploic Appendagitis. Small ovoid fat-attenuation lesion abutting colon with hyperattenuating rim and possible central dot on CT.

072 Esophageal Coin Ingestion. Coin-shaped radiopaque object in esophagus on AP view; coins usually appear en face in esophagus, unlike airway orientation.

073 Focal Nodular Hyperplasia. Homogeneous arterial hyperenhancement with a central T2-bright scar and delayed scar enhancement.

074 Gallbladder Adenomyomatosis. Gallbladder wall thickening with intramural Rokitansky–Aschoff sinuses producing ultrasound comet-tail artifact or MRI pearl-necklace sign.

075 Gallstone Ileus (including Bouveret Syndrome). Rigler pattern: ectopic gallstone, pneumobilia, and bowel obstruction; duodenal impaction in Bouveret syndrome.

076 Gastric Volvulus. Abnormally rotated or upside-down stomach on radiograph/CT; mesenteroaxial form places antrum above gastroesophageal junction.

077 Hepatic Hemangioma. Peripheral discontinuous nodular enhancement with progressive centripetal fill-in; markedly T2-bright liver lesion.

078 Hepatocellular Adenoma. Arterially enhancing liver mass; subtype clues include opposed-phase signal drop (HNF1A) or inflammatory atoll sign.

079 Jejunoileal Atresia. Multiple dilated proximal small-bowel loops with abrupt atretic transition and gasless distal bowel on neonatal radiograph/contrast study.

080 Multiple Magnet Ingestion. Two or more magnets in separate bowel loops or clumped across bowel walls on radiograph, with pressure injury/perforation risk.

081 Omental Infarction. Larger heterogeneous inflamed fat-density lesion within omentum, usually not directly abutting colon, without central dot.

082 Pancreatic Mucinous Cystic Neoplasm. Body/tail multilocular cystic pancreatic mass in a woman, with septa or peripheral calcification and no duct communication.

083 Pancreatic Serous Cystadenoma. Microcystic honeycomb pancreatic mass with central scar or sunburst calcification and no duct communication.

084 Paraduodenal and Transmesenteric Internal Hernia. CT cluster of small-bowel loops in an abnormal sac/compartment with mesenteric vessel crowding or swirl; specify paraduodenal recess versus transmesenteric defect.

085 Pill-Induced Esophagitis. Discrete opposing mid-esophageal ulcers at a pill-stasis site, often with focal mucosal injury on endoscopy.

086 Schatzki Ring. Thin circumferential mucosal ring at the distal esophagus, best seen as a short shelf-like narrowing on barium esophagram.

087 Solid Pseudopapillary Neoplasm. Well-circumscribed encapsulated pancreatic mass with hemorrhagic/cystic center and peripheral solid tissue.

088 Splanchnic venous thrombosis. Portal, mesenteric, or splenic venous filling defects and chronic portal cavernous transformation; include infective pylephlebitis as a subtype.

089 Stercoral Colitis. Fecaloma with colonic dilation and wall thickening plus pericolonic fat stranding, often rectosigmoid, on CT.

090 Superior Mesenteric Artery Syndrome. CT sagittal narrowing of third duodenum between aorta and SMA with proximal gastric/duodenal dilation and reduced aortomesenteric angle/distance.

Next priority 19 topics

091 Biliary Mucinous Cystic Neoplasm. Multiloculated hepatic cyst with septa or mural nodules, usually no bile-duct communication; often in middle-aged women.

092 Congestive Hepatopathy and Fontan-Associated Liver Disease. Heterogeneous “nutmeg” liver enhancement, dilated hepatic veins/IVC and pulsatile portal flow on Doppler in right-sided heart failure/Fontan circulation.

093 Encapsulating peritoneal sclerosis. Encased clustered bowel loops within a thick fibrous peritoneal membrane (“abdominal cocoon”) on CT.

094 Esophageal Intramural Pseudodiverticulosis. Barium esophagram shows numerous tiny flask-shaped intramural outpouchings, sometimes with a stricture.

095 Groove Pancreatitis. Sheet-like soft tissue in pancreaticoduodenal groove with medial duodenal wall thickening or cysts on MRI/CT.

096 Hepatic Epithelioid Hemangioendothelioma. Multiple peripheral hypoenhancing liver nodules coalescing to the capsule, with capsular retraction and possible “lollipop” vessel termination.

097 Hepatic sinusoidal obstruction syndrome. Doppler ultrasound may show hepatomegaly, ascites, reduced/monophasic hepatic venous flow and portal-flow changes after transplant/chemotherapy.

098 Herpes Simplex Esophagitis. Multiple small, sharply demarcated punched-out esophageal ulcers on endoscopy, often distal; biopsy from ulcer edge.

099 Hookworm Infection. Upper endoscopy shows adult hookworms attached to duodenal/jejunal mucosa, sometimes with punctate bleeding.

100 Intestinal duplication cyst. Cyst adjacent to bowel with layered inner echogenic mucosa and outer hypoechoic muscle (“gut signature”) on ultrasound.

101 Isolated Polycystic Liver Disease. Numerous hepatic cysts with relative absence of renal cysts on CT/MRI.

102 Median Arcuate Ligament Syndrome. Respiratory dependent hooked proximal celiac artery narrowing on sagittal CTA or Doppler in a compatible symptomatic syndrome; isolated compression can be incidental.

103 Obturator hernia. Bowel extending through the obturator canal between the obturator and pectineus muscles on pelvic CT.

104 Peritoneal Mesothelioma. Diffuse peritoneal thickening/nodularity, omental cake, or ascites on CT, often without an identifiable gastrointestinal primary.

105 Sclerosing Mesenteritis. Root-of-mesentery misty fat or mass that encases mesenteric vessels while preserving a fat-ring halo; possible pseudocapsule on CT.

106 Secondary Sclerosing Cholangitis of Critical Illness. MRCP/ERCP shows biliary casts, multifocal intrahepatic strictures and pruning after critical illness, initially sparing the extrahepatic duct.

107 Spigelian hernia. Herniation through the Spigelian fascia along the semilunar line on CT or dynamic ultrasound.

108 Undifferentiated Embryonal Sarcoma of the Liver. Large heterogeneous pediatric liver mass that appears cystic on CT but predominantly solid on ultrasound.

109 Yersinia Enterocolitica Ileitis. Terminal ileal wall thickening and mesenteric lymphadenopathy on CT/ultrasound, with a relatively normal appendix in pseudoappendicitis.

Hematology

First priority 3 topics

110 Acquired hemophilia A. Spontaneous skin, muscle, or retroperitoneal hematomas without typical hemarthroses.

111 Post-transplant lymphoproliferative disorder. Post-transplant extranodal allograft, GI, lung, skin, or CNS lesions on PET/CT.

112 Primary mediastinal large B-cell lymphoma. Facial plethora/chest-wall collaterals with bulky anterior mediastinal mass compressing SVC.

Next priority 12 topics

113 EBV-positive mucocutaneous ulcer. Single sharply edged oral or skin ulcer without mass or systemic lymphoma.

114 Enteropathy-associated T-cell lymphoma. Celiac-associated jejunal thickening and aneurysmal dilatation, with ulcers or perforation.

115 Immunoproliferative small intestinal disease (alpha heavy chain). Proximal small bowel thickened folds, cobblestoning, villous edema, and ulcers.

116 Plasmablastic lymphoma. Fungating gingival/palatal mass with lytic maxillary or mandibular lesion.

117 Plasminogen deficiency type 1 (ligneous disease). Wood-like conjunctival membranes and gingival pseudomembranes with tooth loss.

118 Radioulnar synostosis with amegakaryocytic thrombocytopenia. Blocked forearm rotation with bilateral proximal radioulnar synostosis and petechiae.

119 Systemic ALK-positive anaplastic large cell lymphoma. Ulcerated violaceous skin nodules with PET-avid nodal/visceral disease.

120 Systemic chronic active EBV disease. Hemorrhagic bullae, necrosis, and scars precisely at mosquito-bite sites.

121 T-cell prolymphocytic leukemia. Diffuse erythematous papules/plaques, periorbital edema, leonine facies.

122 TEMPI syndrome. Facial/truncal telangiectasias plus perinephric fluid on CT/US and pulmonary shunt.

123 Transient abnormal myelopoiesis. Vesiculopustular lesions in a neonate with Down syndrome, sometimes concentrated at adhesive or venipuncture sites; blood evaluation establishes transient abnormal myelopoiesis.

124 Type I (monoclonal) cryoglobulinemia. Noninflammatory retiform purpura, acral necrosis, and digital/ear gangrene.

Immunology

Next priority 14 topics

125 Adenosine deaminase deficiency (ADA-SCID). Chest radiograph may show absent thymic shadow; characteristic costochondral flaring/cupping ("scorbutic"/rachitic-like) is supportive but not universal.

126 Autoimmune lymphoproliferative syndrome. Generalized cervical/axillary lymphadenopathy with splenomegaly on examination or ultrasound/CT.

127 CARD9 deficiency. Deep dermatophytosis with plaques/nodules or draining abscesses; MRI can show Candida brain abscess.

128 COPA syndrome. Childhood diffuse lung disease with ground glass, cysts, and nodules paired with arthritis.

129 FOXN1 deficiency (nude SCID). Congenital alopecia with eyebrow/lash loss and nail dystrophy on clinical photographs; absent thymic shadow on infant radiograph.

130 IL-10 and IL-10-receptor deficiency. Infantile perianal fistulas/abscesses and deep colonic ulcers on endoscopy.

131 Isolated congenital asplenia. Howell–Jolly bodies on peripheral smear; ultrasound/CT confirms absent spleen with situs solitus.

132 MHC class I deficiency (TAP1/TAP2). Chronic necrotizing skin ulcers and nasal cartilage destruction; bronchiectasis may be seen on chest CT.

133 PLCG2-associated disease (PLAID / APLAID). PLAID can show evaporative cold urticaria and neonatal acral ulcers; APLAID more often has blistering or inflammatory skin lesions without cold induced symptoms.

134 Severe congenital neutropenia. Early severe gingivitis/periodontitis, oral ulcers, and alveolar bone loss.

135 Spondyloenchondrodysplasia with immune dysregulation. Spine radiographs show platyspondyly/metaphyseal lesions; brain CT can show basal-ganglia calcification.

136 Trichohepatoenteric syndrome. Brittle woolly hair with trichorrhexis nodosa on light microscopy, plus characteristic facial appearance.

137 Vici syndrome. Brain MRI shows corpus-callosum agenesis, colpocephaly, and pontine/cerebellar hypoplasia; hypopigmentation/cataracts clinically.

138 WHIM syndrome. Extensive refractory cutaneous/plantar/anogenital warts with recurrent infections.

Nephrology

First priority 6 topics

139 Acute pyelonephritis, including lobar nephronia. Contrast CT may show wedge-shaped hypoenhancement or a striated nephrogram; lobar nephronia appears mass-like.

140 Emphysematous pyelonephritis. CT shows gas in renal parenchyma/collecting system with possible perinephric extension.

141 Horseshoe kidney. CT/urography shows fused lower poles via midline isthmus, low position, and anteriorly rotated renal pelves.

142 Pyonephrosis. Obstructed collecting system with mobile echogenic debris/fluid-debris level on ultrasound, with hydronephrosis.

143 Renal and perinephric abscess. Round or wedge-shaped low-attenuation renal/perinephric collection with enhancing rim on contrast CT.

144 Renal osteodystrophy and secondary hyperparathyroidism. Rugger-jersey spine, salt-and-pepper skull, and subperiosteal resorption in CKD.

Next priority 12 topics

145 Acquired cystic kidney disease. Multiple bilateral renal cysts in small echogenic kidneys on ultrasound/CT in advanced CKD or dialysis.

146 Acute cortical necrosis. Contrast CT shows bilateral cortical hypoenhancement with preserved medullary enhancement (reverse-rim sign); chronic cortical calcification may follow.

147 Bartter syndrome. Bilateral medullary nephrocalcinosis in a neonate with antenatal polyhydramnios.

148 Crossed fused renal ectopia. Both renal moieties lie on one side; trace the crossed ureter to its normal contralateral bladder insertion on CT/urography.

149 Distal (type 1) renal tubular acidosis. Medullary nephrocalcinosis on US/CT; pediatric rickets films may show metaphyseal cupping/fraying.

150 HNF1B related kidney disease. Renal ultrasound shows cysts/dysplasia; abdominal imaging may show hypoplastic or absent pancreatic body/tail.

151 Lithium nephropathy. MRI shows innumerable tiny bilateral cortical and medullary T2-bright microcysts in chronic lithium exposure.

152 Nephronophthisis. Small/normal echogenic kidneys with corticomedullary-junction cysts on ultrasound.

153 Nutcracker syndrome. Left renal vein compression with a beak and gonadal or pelvic collaterals on Doppler or CT, paired with compatible hematuria or pain; distinguish asymptomatic compression.

154 Page kidney. Subcapsular hematoma compresses/flattenes renal parenchyma on CT or ultrasound, with new secondary hypertension.

155 Renal lymphoma. Multiple bilateral homogeneous hypoenhancing renal masses, diffuse enlargement, or perirenal rind on CT/MRI.

156 Renal medullary carcinoma. Infiltrative hypoenhancing central/medullary renal mass in a young patient, often with sickle cell trait and nodal disease.

Neurology

First priority 14 topics

157 Acute flaccid myelitis. Longitudinal gray matter predominant spinal cord T2 signal with acute asymmetric flaccid weakness.

158 Acute ischemic stroke. Territorial diffusion restriction, early CT ischemic changes, and arterial occlusion on CTA.

159 Aneurysmal subarachnoid hemorrhage. Blood in basal cisterns and sulci on CT, with a causative aneurysm on CTA or angiography.

160 Carbon monoxide poisoning. Bilateral globus pallidus injury and delayed white matter abnormalities may appear on MRI.

161 Ependymoma. Intraventricular or spinal mass; posterior fossa tumors may extend through the fourth ventricular outlets.

162 Frontotemporal dementia. Frontal and anterior temporal predominant atrophy or hypometabolism.

163 Guillain Barre syndrome. Facial diplegia or flaccid weakness; smooth cauda equina root enhancement can support an atypical case.

164 Intracranial epidermoid cyst. Extra axial CSF like lesion with marked DWI hyperintensity.

165 Oligodendroglioma. Cortical or subcortical infiltrative mass often containing calcification.

166 Osmotic demyelination syndrome. Symmetric central pontine signal with peripheral sparing, sometimes with extrapontine lesions.

167 Periventricular nodular heterotopia. Gray matter signal nodules lining the lateral ventricles.

168 Polymicrogyria. Excess small gyri with an irregular cortical surface and gray white junction on MRI.

169 Spinal cord infarction. Anterior cord diffusion restriction; sagittal pencil like signal and axial owl eyes may occur.

170 Spinal epidural abscess. Rim enhancing epidural collection with cord or cauda equina compression on MRI.

Next priority 39 topics

171 Acute necrotizing encephalopathy. Symmetric thalamic lesions, sometimes with concentric hemorrhagic or necrotic components.

172 Alpers Huttenlocher syndrome. Posterior cortical and thalamic MRI abnormalities with refractory epilepsy and liver disease.

173 Autoimmune GFAP astrocytopathy. Radial perivascular enhancement extending from the ventricles, with possible cord or meningeal involvement.

174 Balo concentric sclerosis. Alternating concentric rings on T2 and diffusion imaging.

175 Beta propeller protein associated neurodegeneration. Iron sensitive substantia nigra and pallidal signal, with a T1 bright nigral halo in later disease.

176 Biotin thiamine responsive basal ganglia disease. Symmetric caudate and putamen swelling or injury on MRI in acute encephalopathy.

177 Central neurocytoma. Bubbly intraventricular mass near the foramen of Monro and septum pellucidum.

178 Choroid plexus tumors. Frond like avidly enhancing ventricular papilloma versus invasive heterogeneous carcinoma with necrosis or dissemination; hydrocephalus may accompany either.

179 CLIPPERS. Numerous punctate and curvilinear enhancing foci centered on the pons and cerebellar peduncles.

180 COL4A1 related disorders. Porencephaly, small vessel disease, and variable ocular abnormalities such as cataract.

181 CSF1R related leukoencephalopathy. Confluent white matter injury, callosal thinning, persistent diffusion abnormalities, and small CT calcifications.

182 Dyke Davidoff Masson syndrome. Cerebral hemiatrophy with ipsilateral calvarial thickening and sinus enlargement.

183 Dysembryoplastic neuroepithelial tumor. Bubbly multinodular cortical lesion with little mass effect in a patient with epilepsy.

184 Dysferlinopathy. Miyoshi phenotype shows prominent posterior calf wasting, with selective muscle involvement on imaging.

185 Enterovirus A71 brainstem encephalitis. Dorsal brainstem signal with a compatible hand foot and mouth eruption.

186 Familial dysautonomia. Smooth tongue from absent fungiform papillae and absent overflow emotional tears.

187 Ganglioglioma. Superficial cortical tumor, often temporal, sometimes cystic with a mural nodule or calcification.

188 Heroin inhalation associated leukoencephalopathy. Symmetric cerebellar and posterior cerebral white matter abnormalities.

189 Horizontal gaze palsy with progressive scoliosis. Split pons and butterfly shaped medulla with progressive scoliosis.

190 HTRA1 related cerebral small vessel disease. Leukoencephalopathy with lacunes; classic CARASIL adds early alopecia and spondylosis, and an arc sign can involve the pons and middle cerebellar peduncles.

191 Idiopathic spinal cord herniation. Focal ventral cord displacement and kinking through a dural defect on MRI.

192 Infratentorial superficial siderosis. Susceptibility blooming coating cerebellar folia, brainstem, and cranial nerves.

193 Intracranial dermoid cyst. Fat containing intracranial lesion, with disseminated fat droplets after rupture.

194 Japanese encephalitis. Bilateral thalamic lesions, sometimes involving basal ganglia and brainstem.

195 LAMA2 related muscular dystrophy. Congenital weakness or contractures with extensive characteristic white matter signal abnormalities.

196 Listeria rhombencephalitis. Brainstem and cerebellar peduncle inflammation or microabscesses on MRI.

197 Marchiafava Bignami disease. Corpus callosum swelling or injury with selective central layer involvement.

198 Metronidazole induced encephalopathy. Symmetric dentate nucleus abnormalities, sometimes involving the splenium or brainstem.

199 Nasu Hakola disease. Cystic bone lesions paired with frontal predominant white matter degeneration.

200 Neuralgic amyotrophy. Scapular winging with patchy muscle denervation edema or atrophy on MRI.

201 Neuronal intranuclear inclusion disease. Curvilinear DWI hyperintensity along the corticomedullary junction.

202 Pleomorphic xanthoastrocytoma. Superficial hemispheric cyst and mural nodule, sometimes with meningeal contact.

203 Pontocerebellar hypoplasia. Small ventral pons and cerebellum; a dragonfly configuration occurs in some forms.

204 Retinal vasculopathy with cerebral leukoencephalopathy and systemic manifestations. Retinal vascular abnormalities and enhancing white matter lesions that can resemble tumors.

205 Rhombencephalosynapsis. Fusion of cerebellar hemispheres across a deficient vermis.

206 SPG11 related hereditary spastic paraplegia. Thin corpus callosum and ears of the lynx white matter signal near the frontal horns.

207 Spinal arachnoid web. Focal dorsal cord indentation producing a sagittal scalpel sign, sometimes with a syrinx.

208 West Nile neuroinvasive disease. Deep gray, brainstem, or anterior horn abnormalities can accompany encephalitis or acute flaccid paralysis.

209 X linked myotubular myopathy. Long myopathic face, ptosis, ophthalmoparesis, and severe neonatal hypotonia.

Orthopedics

First priority 15 topics

210 Acromioclavicular joint separation. Shoulder step-off and widened coracoclavicular distance on AP/Zanca view.

211 Calcaneus fracture. Lateral heel film with decreased Böhler angle; distinguish tongue-type and joint-depression patterns.

212 Galeazzi fracture-dislocation. Distal radial shaft fracture with distal radioulnar joint disruption.

213 Hangman fracture. CT shows bilateral pars interarticularis fractures of C2 with C2-on-C3 translation/angulation.

214 Jefferson fracture. Axial CT shows bilateral C1 anterior and posterior arch fractures with lateral-mass displacement around the dens.

215 Knee dislocation. Gross deformity or reduced knee with multiligament disruption; assess popliteal artery.

216 Lisfranc injury. Weight-bearing foot radiograph: tarsometatarsal widening or fleck sign.

217 Monteggia fracture-dislocation. Proximal ulna fracture with radial head dislocated on elbow films.

218 Neuropathic arthropathy. Joint fragmentation, debris, disorganization, and collapse; rocker bottom foot in advanced midfoot disease.

219 Odontoid fracture. Open-mouth and lateral cervical films/CT show fracture through C2 dens (type I tip, type II base, type III body).

220 Osgood-Schlatter disease. Prominent, fragmented tibial tubercle on lateral knee radiograph.

221 Patellar dislocation. Sunrise view shows lateral patellar displacement; MRI may show MPFL injury and contusions.

222 Radial head fracture. Elbow radiograph shows radial-head cortical break or depression; a posterior fat-pad sign can disclose an occult fracture.

223 Supracondylar humerus fracture. Elbow radiograph: posterior fat pad, altered Baumann angle, or anterior humeral line; puckered skin can indicate entrapment.

224 Traumatic hip dislocation. Shortened flexed, adducted, internally rotated leg; AP pelvis shows posteriorly displaced femoral head.

Next priority 77 topics

225 Acetabular fracture. Pelvic oblique/Judet views or CT show fracture through acetabular columns/walls with disruption of iliopectineal or ilioischial lines.

226 Adhesive capsulitis. Marked loss of passive external rotation; MRI may show thickened coracohumeral ligament and edema/obliteration of rotator interval fat.

227 Adverse local tissue reaction (metallosis). Metal-on-metal hip MRI with metal-artifact reduction shows periarticular pseudotumor, synovitis, or fluid collection; radiograph may show metal debris.

228 Atelosteogenesis. Severe limb shortening with hypoplastic or poorly ossified long bones and characteristic joint or skeletal abnormalities; include boomerang phenotype as a labeled variant.

229 Baastrup disease. Lateral lumbar radiograph/CT shows close contact and sclerosis of adjacent spinous processes; MRI may show interspinous bursitis/edema.

230 Biceps tendon rupture. Clinical Popeye bulge and palpable tendon gap; ultrasound/MRI shows proximal long-head tendon discontinuity or distal avulsion/retraction.

231 Cervical disc herniation. MRI shows focal cervical disc extrusion/protrusion indenting ventral thecal sac or compressing a nerve root/spinal cord.

232 Chance fracture. Lateral spine CT shows a horizontal flexion-distraction fracture through vertebral body and posterior elements, often with a seatbelt injury.

233 Chronic osteomyelitis. Sequestrum, involucrum, cloaca, and sinus tract on radiograph/CT/MRI.

234 Clay-shoveler fracture. Lateral cervical/thoracic radiograph shows avulsion fracture of a lower cervical or upper thoracic spinous-process tip.

235 Congenital knee dislocation. Newborn knee rests in hyperextension; lateral radiograph/US shows anterior tibial displacement relative to distal femur.

236 Congenital pseudarthrosis of the clavicle. Visible painless midclavicular prominence; radiograph shows a gap/nonunion, usually right midshaft, with tapered ends.

237 Cubital tunnel syndrome. Ulnar-distribution paresthesia and intrinsic hand weakness; ultrasound/MRI shows enlarged ulnar nerve at cubital tunnel, sometimes with dynamic subluxation.

238 de Quervain tenosynovitis. Radial-styloid pain with positive Finkelstein/Eichhoff maneuver; US shows thickened first dorsal compartment tendon sheath.

239 Discoid meniscus. MRI shows abnormally broad meniscus and continuity of anterior/posterior horns across at least three sagittal images (bow-tie sign).

240 Elastofibroma dorsi. Deep infrascapular lentiform mass abutting chest wall; MRI shows muscle-like fibrous signal interlaced with linear fat streaks, often bilateral.

241 Essex-Lopresti injury. Radial-head fracture with proximal radial migration and distal radioulnar joint widening/positive ulnar variance; image elbow, forearm, and wrist.

242 Extensor mechanism rupture. Palpable tendon gap with patella alta (patellar tendon) or baja (quadriceps) on lateral radiograph.

243 Fight bite. Small dorsal MCP laceration over a clenched-fist knuckle with swelling; radiograph may show tooth fragment or joint/phalangeal injury.

244 Freiberg infraction. Radiograph shows flattening, sclerosis, and subchondral collapse of a metatarsal head (usually second) in an adolescent.

245 Frontometaphyseal dysplasia. Prominent supraorbital ridges and craniofacial hyperostosis with metaphyseal flaring, long-bone bowing, and digital contractures.

246 Grisel syndrome. CT/MRI shows nontraumatic atlantoaxial rotatory subluxation in a child after upper-airway infection or surgery.

247 Haglund syndrome. Lateral heel film shows posterosuperior calcaneal prominence with retrocalcaneal bursal fluid and distal Achilles insertional thickening.

248 Hydroxyapatite deposition disease. Periarticular calcific cloud in rotator-cuff tendon on shoulder radiograph.

249 Incarcerated medial epicondyle. After elbow dislocation, radiograph shows medial epicondylar fragment trapped in the joint space rather than at its normal medial location.

250 Intramuscular myxoma. MRI shows a well-circumscribed intramuscular T1-low/T2-high mass with perilesional fat rind or edema and variable peripheral enhancement.

251 Intraosseous lipoma. Well-defined intramedullary lucency with sclerotic rim and central dystrophic calcification; CT/MRI confirms fat attenuation/signal.

252 Isolated congenital radioulnar synostosis. Forearm radiograph shows proximal radius-ulna bony bridge with fixed pronation; distinguish from syndromic synostosis.

253 Jansen metaphyseal chondrodysplasia. Short stature with metaphyseal widening, cupping/fraying, and cortical erosion.

254 Jersey finger. Inability to flex DIP after forced extension; sagittal MRI/US shows FDP avulsion from distal phalanx with proximal tendon retraction.

255 Jones fracture. Oblique foot radiograph shows fracture at the metaphyseal-diaphyseal junction of the fifth metatarsal, extending into the fourth-fifth intermetatarsal articulation (zone 2).

256 Köhler disease. Child with midfoot pain; foot radiograph shows flattened, dense, fragmented navicular that later reossifies.

257 Lateral condyle fracture. Pediatric elbow radiograph shows lateral distal-humeral metaphyseal fragment extending through the unossified articular cartilage; internal oblique view improves displacement assessment.

258 Lesser-toe deformities. Inspection shows fixed or flexible hammertoe (PIP flexion), mallet toe (DIP flexion), or claw toe (MTP hyperextension with IP flexion); weight-bearing film defines alignment.

259 Lipoma arborescens. Knee MRI shows frond-like synovial projections with T1/T2 signal matching fat and suppression on fat-saturated sequences, often with effusion.

260 Lumbar facet synovial cyst. MRI shows a fluid-signal juxtafacet cyst communicating with degenerative lumbar facet and narrowing lateral recess/canal.

261 Medial collateral ligament tear. Valgus stress tenderness/laxity; MRI shows edema and thickening or fiber discontinuity of the MCL, commonly with medial femoral condyle contusion.

262 Morel-Lavallée lesion. MRI shows a lentiform hemolymphatic collection with fat globules in the plane between subcutaneous tissue and deep fascia, often over greater trochanter.

263 Morton neuroma. Forefoot squeeze/Mulder click; US/MRI shows plantar interdigital nerve thickening in a web space, classically third-fourth.

264 Myxofibrosarcoma. MRI of a superficial lower-extremity mass shows high T2 myxoid signal and curvilinear enhancing fascial tails.

265 Müller-Weiss disease. Adult midfoot radiographs show comma-shaped navicular collapse/sclerosis with lateral displacement and talonavicular arthritis.

266 Neurogenic heterotopic ossification. After spinal cord/brain injury, radiograph/CT shows periarticular ectopic bone bridging soft tissues, commonly around hip.

267 Nora lesion. Radiograph shows a mineralized surface-based exostosis on a small bone of hand/foot, separated from medullary cavity; may have a cartilage cap.

268 Ossification of the ligamentum flavum. CT shows posterior ligamentum flavum ossification, often thoracic; MRI demonstrates posterior canal narrowing and cord compression.

269 Panner disease. Child with elbow pain; radiograph/MRI shows capitellar epiphyseal sclerosis, fragmentation, and flattening.

270 Particle-disease osteolysis. Serial arthroplasty films show focal expansile periprosthetic osteolytic scalloping/osteolysis around wear-bearing surfaces.

271 Patella fracture. AP/lateral/sunrise knee films show transverse, vertical, or comminuted patellar fracture and possible articular step-off; assess extensor mechanism.

272 Pectoralis major rupture. Acute chest/axillary ecchymosis with loss of anterior axillary fold; MRI shows tendon avulsion or retracted muscle-tendon unit.

273 Pediatric incomplete forearm fractures. Pediatric forearm radiographs show torus cortical buckling or greenstick fracture with one cortex broken and the opposite cortex bowed/intact.

274 Pelvic apophyseal avulsions. Pelvic radiographs show a displaced apophyseal fragment at the ASIS, AIIS, ischial tuberosity, or lesser trochanter; MRI shows edema when the apophysis is unossified.

275 Pelvic ring disruption. AP/inlet/outlet pelvis views or CT show ring discontinuity, sacroiliac widening, pubic-ramus displacement, or vertical hemipelvic shift.

276 Perilunate and lunate dislocation. Lateral wrist film with spilled-teacup lunate and disrupted carpal arcs.

277 Periosteal chondroma. Small cortical-surface cartilage mass with saucer-like cortical erosion, sclerotic rim, and overhanging edges on radiograph/CT.

278 Periprosthetic joint infection. Painful prosthesis with sinus tract or effusion; imaging can show periprosthetic lucency/loosening, while aspiration and cultures confirm infection.

279 Peroneal tendon dislocation. Dynamic US during resisted eversion shows peroneal tendons snapping anterior to fibula due to superior retinaculum injury; MRI depicts retinaculum tear.

280 Plantar fibromatosis. Palpable medial plantar-arch nodule; US/MRI shows nodular thickening arising from the plantar aponeurosis, usually along its medial band.

281 Posterior cruciate ligament tear. Posterior sag on exam; MRI shows PCL discontinuity or dashboard contusion pattern.

282 Posterolateral corner injury. MRI shows edema or discontinuity of fibular collateral ligament, popliteus tendon, and popliteofibular ligament; fibular-head arcuate avulsion is a clue.

283 Primary lymphoma of bone. Radiograph/MRI shows a permeative marrow lesion with a large soft-tissue mass despite relatively limited cortical destruction.

284 Pyle disease. Radiographs show marked metaphyseal expansion of long bones with thin cortices (Erlenmeyer-flask shape), especially around knees.

285 Pyogenic spondylodiscitis. MRI shows disc-space signal and adjacent endplate destruction.

286 Pyomyositis. MRI shows muscle edema progressing to a rim-enhancing intramuscular abscess with central fluid signal; US can guide drainage.

287 Scapholunate dissociation. PA wrist radiograph shows widened scapholunate interval (Terry-Thomas sign) and scaphoid ring sign; lateral view may show DISI.

288 Septic bursitis. US/MRI shows distended olecranon or prepatellar bursa with thick hyperemic wall, echogenic debris, and surrounding cellulitis.

289 Sinding-Larsen-Johansson disease. Adolescent with inferior patellar-pole tenderness; lateral knee film shows fragmentation/ossicle at the patellar tendon origin.

290 Skeletal metastases. Lytic missing pedicle, ivory vertebra, or permeative metadiaphyseal lesion.

291 Split-hand/foot malformation. Clinical/radiographic central ray deficiency creates median cleft with absent central metacarpals/metatarsals and divergent digits.

292 Subchondral insufficiency fracture of the knee. MRI shows subchondral low-signal fracture line with extensive marrow edema, usually medial femoral condyle; early radiographs may be normal.

293 Talar neck fracture. Ankle lateral radiograph/CT shows talar neck fracture; Hawkins classification records subtalar/tibiotalar displacement, and the later Hawkins sign is subchondral talar-dome lucency.

294 Terrible triad of the elbow. Elbow films/CT show posterior dislocation with radial-head and coronoid-process fractures.

295 Thumb ulnar collateral ligament injury. Thumb MCP valgus laxity; MRI/US shows UCL discontinuity, retraction, or Stener lesion superficial to the adductor aponeurosis.

296 Tibial hemimelia. AP leg radiograph shows partial/complete tibial absence or hypoplasia with fibular bowing and knee/ankle deformity.

297 Tibial tubercle avulsion. Lateral knee radiograph shows displaced tibial tubercle fragment in an adolescent; inspect for patella alta and extensor-mechanism disruption.

298 Transient osteoporosis of the hip. Diffuse femoral head and neck marrow edema, later osteopenia, without the typical focal necrotic boundary.

299 Transitional ankle fractures. CT distinguishes Tillaux fracture of the anterolateral distal tibial epiphysis from triplane fracture involving epiphysis, physis, and metaphysis in three planes.

300 Ulnar longitudinal deficiency. Forearm/hand radiographs show hypoplastic or absent ulna with radial bowing and ulnar-sided digit deficiency.

301 Vertebral hemangioma. Corduroy trabeculae on radiograph and polka-dot trabeculae on axial CT.

Otolaryngology

First priority 18 topics

302 Carotid body tumor. Hypervascular carotid bifurcation mass splaying ICA and ECA (lyre sign) on CTA/MRA.

303 Congenital laryngeal web. Thin anterior glottic web visible at laryngoscopy, sometimes with a small anterior notch.

304 Idiopathic subglottic stenosis. Short circumferential fibrotic narrowing immediately below the glottis on endoscopy/CT.

305 Laryngeal cleft. Posterior interarytenoid cleft on direct laryngoscopy/bronchoscopy; aspiration on swallow study.

306 Laryngeal fracture. CT cartilage discontinuity or laryngeal framework displacement with laryngoscopic mucosal injury/airway edema.

307 Le Fort fractures (types I–III as one topic). CT shows Le Fort fracture line crossing the maxilla/pterygoid plates; classify I, II, or III by separation level.

308 Mandibular fracture. Mandibular cortical discontinuity on panoramic radiograph/CT; assess symphysis, body, angle, and condyle.

309 Nasal bone fracture. Displaced or comminuted nasal-bone fracture on facial CT, with bridge deviation/swelling clinically.

310 Nasal foreign body, including a button battery. Unilateral nasal discharge with visible foreign body; button battery may produce septal ulceration/perforation.

311 Nasal septal hematoma. Boggy, fluctuant bilateral septal swelling obstructing both nasal passages after trauma.

312 Orbital blowout fracture. Orbital floor defect with herniated orbital fat/inferior rectus and maxillary hemosinus on coronal CT.

313 Parapharyngeal abscess. Rim-enhancing collection in parapharyngeal space on contrast CT, with airway displacement.

314 Sinonasal fungal ball. Single opacified sinus containing central hyperattenuating/calcific material with reactive wall sclerosis on CT.

315 Superior semicircular canal dehiscence syndrome. Canal plane temporal bone CT shows a superior canal roof defect, paired with compatible sound or pressure induced symptoms and physiologic findings.

316 Temporal bone fracture (otic-capsule sparing versus violating). High-resolution temporal-bone CT shows fracture line and whether it crosses otic capsule; associated hemotympanum/CSF leak.

317 Tympanosclerosis. Discrete chalk-white plaques in the tympanic membrane, often with reduced membrane mobility.

318 Vallecular cyst. Smooth cyst arising in vallecula, displacing epiglottis posteriorly on flexible laryngoscopy.

319 Zygomaticomaxillary complex fracture. CT shows zygomatic arch depression, widened lateral orbit, and disruption of zygomaticomaxillary/orbital rims.

Next priority 20 topics

320 Acute bacterial sialadenitis. Unilateral tender swollen salivary gland with purulent drainage at duct orifice.

321 Cholesterol granuloma of the petrous apex. Petrous-apex lesion with high signal on both T1- and T2-weighted MRI and smooth expansion.

322 Chronic invasive fungal rhinosinusitis. Unilateral infiltrative sinonasal soft tissue with bone destruction and extrasinus extension on CT/MRI.

323 Cocaine-induced midline destructive lesion. Midline septal, turbinate, and palatal ulceration with cartilage/bone destruction; palate perforation in advanced disease.

324 Enlarged vestibular aqueduct. Enlarged vestibular aqueduct on high-resolution temporal-bone CT, measured at midpoint; commonly bilateral.

325 External auditory canal cholesteatoma. Focal canal-floor soft-tissue erosion with sequestrated bone on temporal-bone CT; keratin debris on otoscopy.

326 External auditory canal osteoma. Single dense pedunculated bony projection from the external auditory canal on otoscopy/CT.

327 First branchial cleft anomaly. Periauricular pit/cyst or draining sinus with tract through/parotid region toward external auditory canal.

328 Keratosis obturans. Dense keratin plug filling an expanded external canal, with severe otalgia and conductive hearing loss.

329 Nasolabial (nasoalveolar) cyst. Smooth cystic swelling at the nasal ala/vestibule, typically below the piriform aperture on CT/MRI.

330 Odontogenic sinusitis. Unilateral maxillary sinus opacification contiguous with a diseased maxillary tooth or dental implant on CT.

331 Petrous apicitis (Gradenigo syndrome). Petrous-apex opacification/erosion on temporal-bone CT with adjacent apical inflammation; diplopia from CN VI palsy.

332 Preauricular sinus. Small pit immediately anterior to the ascending helix; expressible discharge when infected.

333 Reinke edema. Bilateral diffuse gelatinous/polypoid swelling of the true vocal folds on laryngoscopy.

334 Sinonasal osteoma. Well-defined ivory-density lesion attached to a frontal or ethmoid sinus wall on CT.

335 Sinonasal squamous cell carcinoma. Irregular enhancing sinonasal mass with focal bone destruction and orbital/skull-base extension on CT/MRI.

336 Vocal fold cyst. Smooth subepithelial vocal-fold cyst causing focal bulge and impaired mucosal wave on stroboscopy.

337 Vocal fold nodules. Bilateral symmetric callus-like lesions at the junction of anterior and middle thirds of true vocal folds on laryngoscopy.

338 Vocal fold polyp. Unilateral focal pedunculated or sessile lesion on the free edge of a true vocal fold.

339 Vocal process granuloma. Posterior vocal-process granuloma at the arytenoid vocal process on laryngoscopy.

Pediatrics

First priority 11 topics

340 Erythema Toxicum Neonatorum. Well newborn has blanching erythematous macules/papules with central papules or pustules, mainly trunk and proximal limbs, sparing palms and soles.

341 Germinal matrix and intraventricular hemorrhage of prematurity. Caudothalamic groove hemorrhage and intraventricular blood on cranial ultrasound.

342 Hair thread tourniquet syndrome. A constricting hair or thread with distal swelling or ischemia on a digit, toe, or penis; include genital presentations within one topic.

343 Hydranencephaly. Near absence of cerebral hemispheres replaced by fluid, with preserved falx and variable deep structures.

344 Kernicterus. Bilateral pallidal signal abnormalities, with chronic T2 changes; early bilirubin injury can be T1 bright.

345 Neonatal mastitis. Unilateral erythematous, warm, tender neonatal breast bud; US fluid collection if abscess.

346 Periventricular leukomalacia. Periventricular cysts or white matter volume loss with irregular ventricular margins.

347 Physical child abuse. Classic metaphyseal lesions, suspicious rib fracture patterns, and intracranial injury; interpret the entire clinical context rather than treating an isolated sign as proof.

348 Pleuropulmonary blastoma. In a young child, CT shows a multiseptated cystic or mixed cystic-solid lung mass, sometimes with pneumothorax, in a DICER1 context.

349 Subgaleal Hemorrhage. Diffuse boggy scalp collection crosses sutures and midline, with subaponeurotic fluid on ultrasound; unlike suture-confined cephalohematoma.

350 Umbilical Granuloma. Small moist beefy-red tissue at the umbilical stump after cord separation, distinguishable from a patent urachus or omphalomesenteric duct.

Next priority 59 topics

351 3M Syndrome. Severe prenatal/postnatal growth restriction with slender long bones, tall vertebral bodies, and characteristic vertebral endplate changes.

352 Alpha-Mannosidosis. Coarse facies, hearing loss, hepatosplenomegaly, and dysostosis multiplex on skeletal radiographs.

353 Asymmetric Periflexural Exanthem of Childhood. Unilateral/periflexural papular erythematous eruption begins near an axilla or groin and spreads centrifugally, often becoming bilateral but asymmetric.

354 ATR-X Syndrome. Characteristic hypotonic facies with upturned nose, tented upper lip, widely spaced teeth, and genital anomalies; HbH inclusions can support diagnosis.

355 Atypical Teratoid/Rhabdoid Tumor. Large heterogeneous infant CNS mass with possible hemorrhage, cysts, restricted diffusion, and hydrocephalus; posterior-fossa lesions can mimic medulloblastoma.

356 Beare-Stevenson Cutis Gyrata Syndrome. Cutis gyrata with acanthosis nigricans, craniosynostosis, prominent umbilical stump, and characteristic facial appearance.

357 Brachytelephalangic Chondrodysplasia Punctata. Short distal phalanges with punctate epiphyseal calcifications and midface/nasal hypoplasia on radiographs.

358 Branchio-Oculo-Facial Syndrome. Branchial skin defects, ocular coloboma/microphthalmia, and cleft lip or prominent philtral pillars with a broad nasal tip.

359 CHILD Syndrome. Strictly unilateral, sharply demarcated ichthyosiform inflammatory nevus follows Blaschko lines with ipsilateral limb hypoplasia/reduction.

360 Cohen Syndrome. Thick hair and eyebrows, downslanting palpebral fissures, open-mouth appearance, joint hypermobility, and later retinal dystrophy.

361 Congenital High Airway Obstruction Syndrome (CHAOS). Prenatal ultrasound/MRI shows enlarged echogenic lungs, dilated distal airways, flattened/inverted diaphragm, and fetal hydrops from laryngeal/tracheal obstruction.

362 Congenital Infantile Fibrosarcoma. A rapidly growing infant soft-tissue mass is often heterogeneous and avidly enhancing on MRI, with hemorrhage or necrosis; imaging overlaps vascular tumors and needs pathology correlation.

363 Currarino Syndrome. Lateral sacral radiograph shows hemisacrum/sickle-shaped sacrum; MRI/US depicts presacral mass with anorectal malformation.

364 Desbuquois Dysplasia. Short limbs and joint laxity with “Swedish key” proximal femoral appearance and vertebral coronal clefts on radiographs.

365 Donnai-Barrow Syndrome. Characteristic facial appearance with high myopia/retinal detachment, sensorineural hearing loss, and diaphragmatic/omphalocele defects.

366 Feingold Syndrome. Microcephaly, short palpebral fissures, brachymesophalangy with clinodactyly, toe syndactyly, and gastrointestinal atresia.

367 Femoral Hypoplasia–Unusual Facies Syndrome. Bilateral or asymmetric femoral hypoplasia with upslanting palpebral fissures, short nose, long philtrum, and cleft palate.

368 Fetal Hydantoin Syndrome. Prenatal phenytoin exposure with nail/digital hypoplasia, midface hypoplasia, cleft lip/palate, and growth restriction.

369 Fetal Valproate Syndrome. Prenatal valproate exposure may produce characteristic facial features, metopic ridge, radial-ray defects, and neural-tube defects.

370 Fryns Syndrome. Coarse facial appearance, distal nail hypoplasia, and diaphragmatic hernia on prenatal or neonatal imaging.

371 Fucosidosis. Progressive coarse facial features and skeletal dysostosis with possible angiokeratomas and white-matter changes.

372 Galactosialidosis. Coarse facies, cherry-red spots, skeletal dysplasia, and vacuolated cells in infantile disease.

373 Grebe Acromesomelic Dysplasia. Marked acromesomelic limb shortening with severe hand/foot abnormalities and characteristic short, broad tubular bones.

374 Greig Cephalopolysyndactyly Syndrome. Macrocephaly, hypertelorism, preaxial polydactyly, and cutaneous syndactyly.

375 Hennekam Syndrome. Lymphedema with characteristic coarse facial appearance, intestinal lymphangiectasia, and possible lymphatic malformations on imaging.

376 Infantile Acropustulosis. Recurrent intensely pruritic crops of sterile vesiculopustules favoring palms and soles, often starting in early infancy.

377 Isotretinoin Embryopathy. Craniofacial dysmorphism with microtia/ear canal anomalies, conotruncal cardiac defects, and thymic/CNS abnormalities after exposure.

378 L1 syndrome. Prenatal/neonatal imaging shows aqueductal stenosis with hydrocephalus; examination shows adducted thumbs.

379 Lipoblastoma. Infant/toddler has a rapidly enlarging soft-tissue mass; MRI shows lobulated fat-containing lesion with septa and variable nonfatty components.

380 LUMBAR Syndrome. Segmental lower-body infantile hemangioma with underlying lumbosacral, urogenital, anorectal, or renal anomalies on MRI/US.

381 Marshall-Smith Syndrome. Progeroid facies with prominent eyes, upturned nose, broad forehead, accelerated skeletal maturation, and respiratory/airway problems.

382 Matthew Wood syndrome. Anophthalmia or severe microphthalmia with diaphragmatic hernia, pulmonary hypoplasia, and cardiac/urogenital anomalies.

383 Meconium Plug Syndrome. Contrast enema shows a normal-caliber colon with long filling defect from meconium; plug may evacuate during the study.

384 Meier-Gorlin Syndrome. Microtia, patellar hypoplasia/absence, and severe pre- and postnatal growth restriction.

385 Melanotic Neuroectodermal Tumor of Infancy. Rapidly enlarging infant anterior maxillary mass with an expansile lytic lesion and variable spiculated/sunburst bone change on radiographs or CT.

386 Melnick-Needles Syndrome. Distinctive facial appearance with slender bowed long bones, cortical irregularity, and rib/spine/pelvic abnormalities.

387 Mesenchymal hamartoma of the liver. Large multicystic infant liver mass with thin septa and intervening solid stromal tissue on ultrasound/CT.

388 Microcephalic osteodysplastic primordial dwarfism type II. Severe microcephaly and primordial dwarfism with characteristic skeletal changes including hypoplastic iliac wings and abnormal metaphyses.

389 Miller Syndrome. Postaxial limb-ray deficiency with malar hypoplasia, micrognathia, and cup-shaped ears.

390 Molybdenum Cofactor Deficiency. Neonatal seizures with MRI diffusion restriction and evolving cystic encephalomalacia involving cortex and subcortical white matter.

391 Muenke Syndrome. Coronal craniosynostosis with variable hypertelorism, brachydactyly/carpal-tarsal fusion, and hearing loss; CT demonstrates the fused coronal suture.

392 Multiple Sulfatase Deficiency. Coarse features, ichthyosis, skeletal dysplasia/dysostosis multiplex, and neurologic decline.

393 Myhre Syndrome. Short stature with thickened skin, brachydactyly, joint stiffness, and radiographic broad short tubular bones/vertebral changes.

394 Neonatal Adrenal Hemorrhage. Ultrasound shows an avascular suprarenal mass that liquefies and shrinks on serial imaging; chronic lesions may calcify.

395 Neonatal Small Left Colon Syndrome. Contrast enema shows a small descending/sigmoid colon with abrupt caliber transition at the splenic flexure and a normal-caliber rectum.

396 Nicolaides-Baraitser Syndrome. Sparse scalp hair, prominent interphalangeal joints, broad fingertips, triangular face, and thick lower lip.

397 Opsismodysplasia. Severe short-limb dwarfism with delayed ossification, small irregular epiphyses, and platyspondyly on skeletal survey.

398 Pallister Hall syndrome. Postaxial polydactyly, bifid epiglottis, and hypothalamic hamartoma on MRI.

399 Pallister-Killian Syndrome. Coarse facial gestalt, temporal alopecia, pigmentary mosaicism, and hypotonia; mosaic tetrasomy 12p may be missed in blood.

400 Pediatric Cystic Nephroma. Renal ultrasound/CT/MRI shows a well-circumscribed multiloculated cystic renal mass with enhancing septa and no solid nodules.

401 Perlman Syndrome. Fetal/neonatal overgrowth with visceromegaly, renal abnormalities/nephroblastomatosis, and characteristic facial appearance.

402 Pitt-Hopkins Syndrome. Wide mouth with full lips, deep-set eyes, broad nasal bridge, and characteristic intermittent hyperventilation/apnea in a child with severe developmental delay.

403 Rhabdoid Tumor of the Kidney. Large heterogeneous infant renal mass with necrosis or hemorrhage; subcapsular fluid or calcification may occur, with a recognized association with CNS rhabdoid tumors.

404 Roberts Syndrome. Severe symmetric tetraphocomelia or limb reduction with cleft lip/palate, microcephaly, and characteristic facial dysmorphism.

405 Schinzel-Giedion Syndrome. Midface retrusion, proptosis, depressed nasal bridge, genital/renal anomalies, and a distinctive skull/vertebral radiographic pattern.

406 Stüve-Wiedemann Syndrome. Bowed long bones with cortical thickening at the concavity, metaphyseal abnormalities, and recurrent fractures.

407 Warfarin Embryopathy. Nasal hypoplasia with stippled epiphyses/vertebrae on skeletal radiographs after first-trimester exposure.

408 Wiedemann Rautenstrauch syndrome. Neonatal progeroid facies, natal teeth, sparse hair, and lipoatrophy.

409 Wiedemann-Steiner Syndrome. Thick flared eyebrows, long eyelashes, downslanting narrow palpebral fissures, and hypertrichosis cubiti (“hairy elbows”).

Pulmonology

First priority 10 topics

410 Airway foreign body aspiration. Unilateral expiratory air trapping/hyperinflation on chest film; foreign body seen on bronchoscopy.

411 Asbestos related nonmalignant thoracic disease. Basal subpleural fibrosis for asbestosis; pleural plaques, diffuse pleural thickening, and associated rounded atelectasis as distinct nonmalignant manifestations.

412 Chronic Thromboembolic Pulmonary Hypertension. V/Q scan shows segmental mismatched perfusion defects; CTPA shows eccentric organized thrombus, webs/bands, pouch defects, mosaic perfusion, and enlarged bronchial arteries.

413 Constrictive Bronchiolitis. Expiratory HRCT shows patchy lobular air trapping/mosaic attenuation, sometimes with bronchiectasis and bronchial-wall thickening.

414 Fat Embolism Syndrome. After long-bone injury, conjunctival/axillary petechiae; chest CT may show diffuse or patchy ground-glass opacity, septal thickening, and centrilobular nodules.

415 Idiopathic Nonspecific Interstitial Pneumonia. HRCT shows lower-lung-predominant ground-glass opacity or fine reticulation with relative subpleural sparing; traction bronchiectasis if fibrotic.

416 Mature Mediastinal Teratoma. Anterior mediastinal mass with macroscopic fat, fluid, soft tissue, and coarse calcification/teeth; fat-fluid level may be present.

417 Rounded Atelectasis. Pleural-based rounded mass adjacent to thickened pleura, with converging bronchovascular structures curving into the lesion (comet-tail sign).

418 Thymoma. CT shows a well-circumscribed or lobulated anterior mediastinal soft-tissue mass; pleural "drop" implants can indicate dissemination.

419 Tracheobronchomalacia. Dynamic expiratory collapse of the tracheal lumen on bronchoscopy or dynamic CT.

Next priority 29 topics

420 Alveolar Macrophage Pneumonia (formerly Desquamative Interstitial Pneumonia). HRCT typically shows diffuse/basal ground-glass opacity in a smoker, sometimes with subpleural sparing and emphysema.

421 Amiodarone Pulmonary Toxicity. HRCT may show peripheral ground-glass opacity or consolidation; high-attenuation lung opacities and high-attenuation liver/spleen support iodine-rich drug accumulation.

422 Bronchial Anthracofibrosis. Bronchoscopy shows black anthracotic pigmentation with bronchial narrowing/stenosis, often involving the right middle lobe; CT may show calcified nodes and atelectasis.

423 Bronchial Atresia. CT shows a blind-ending bronchus with a mucocele/bronchocele and adjacent segmental hyperinflation/air trapping.

424 Bronchial Mucoepidermoid Carcinoma. Central endobronchial soft-tissue nodule/mass on CT causing focal airway narrowing, atelectasis, or recurrent postobstructive infection.

425 Bronchogenic Cyst. Smooth, sharply marginated water-attenuation mediastinal (often subcarinal) or intrapulmonary cyst on CT; proteinaceous contents may increase attenuation.

426 Combined Pulmonary Fibrosis and Emphysema. HRCT shows upper-lobe centrilobular/paraseptal emphysema with lower-lobe subpleural fibrosis/honeycombing in the same patient.

427 Complete Tracheal Rings. Axial CT/bronchoscopy shows a circular O-shaped trachea lacking the posterior membranous wall, often with long-segment stenosis.

428 Dendriform Pulmonary Ossification. Thin branching, bone-density opacities in dependent lower-lobe subpleural/interstitial lung on CT, often incidentally detected.

429 Diaphragmatic Eventration. Smooth elevated intact hemidiaphragm with preserved continuity on CT/ultrasound; dynamic ultrasound can show reduced excursion.

430 Diffuse Idiopathic Pulmonary Neuroendocrine Cell Hyperplasia (DIPNECH). HRCT shows diffuse mosaic attenuation/air trapping with multiple small centrilobular nodules and occasional tumorlets or carcinoids.

431 Genetic surfactant dysfunction. Diffuse ground glass, septal thickening, and variable cystic change in term neonates or children; label SFTPB, SFTPC, ABCA3, and NKX2-1 subtypes and do not infer genotype from CT.

432 Inert metal pneumoconiosis. Dense fine pulmonary nodules after iron, tin, or barium dust exposure; keep siderosis, stannosis, and baritosis as labeled etiologic sections.

433 Metastatic Pulmonary Calcification. CT shows upper-lobe-predominant centrilobular ground-glass nodules or calcified nodules in chronic renal failure/hyperparathyroidism; bone scintigraphy may show diffuse pulmonary uptake.

434 Neuroendocrine cell hyperplasia of infancy. HRCT shows geographic ground-glass opacity centered in right middle lobe and lingula with central/paramediastinal distribution and mosaic air trapping.

435 Posterior Mediastinal Schwannoma. Well-circumscribed paravertebral/posterior mediastinal mass, often with smooth neural-foraminal widening or dumbbell extension on CT/MRI.

436 Proximal Interruption of a Pulmonary Artery. CT angiography shows absent proximal branch pulmonary artery with distal intrapulmonary branches supplied by systemic collaterals and a small ipsilateral lung.

437 Pulmonary agenesis and aplasia. Absent or rudimentary ipsilateral main bronchus with absent lung tissue and vasculature, ipsilateral volume loss, and compensatory contralateral expansion.

438 Pulmonary Artery Intimal Sarcoma. CTPA shows a lobulated, expansile filling defect with vessel enlargement/wall-eclipsing; MRI shows heterogeneous enhancement and PET may be FDG avid.

439 Pulmonary Light-Chain Deposition Disease. CT may show multiple cysts and nodules, often peribronchovascular, with airway-wall thickening or calcification.

440 Pulmonary Sclerosing Pneumocytoma. Usually a well-circumscribed, peripherally located, round/oval nodule with homogeneous enhancement; marginal pseudocapsule or overlying-vessel sign may appear.

441 Pulmonary Talcosis. CT after intravenous injection of talc containing crushed tablets may show centrilobular micronodules, high attenuation deposits, and later fibrosis or emphysema.

442 Pulmonary Vein Stenosis. CTA/MRA shows focal pulmonary-vein narrowing or abrupt cutoff with regional septal thickening, ground-glass edema, waxing/waning opacities, or decreased perfusion.

443 Respiratory Bronchiolitis-Interstitial Lung Disease (RB-ILD). Smoker with ill-defined upper-lobe centrilobular nodules, patchy ground-glass opacity, bronchial-wall thickening, and often emphysema on HRCT.

444 Spontaneous Pneumomediastinum. Chest radiograph/CT shows mediastinal air outlining the heart, great vessels, or trachea, sometimes with continuous-diaphragm sign.

445 Thoracic Splenosis. Multiple left pleural-based nodules on CT after remote splenic trauma/splenectomy; lesions enhance like splenic tissue and show uptake on heat-damaged RBC scan.

446 Tracheobronchial Adenoid Cystic Carcinoma. CT shows smooth submucosal tracheal or main-bronchial wall thickening/narrowing, sometimes with longitudinal extension; bronchoscopy shows a submucosal lesion.

447 Vaping-Associated Lung Injury (EVALI). Chest CT commonly shows bilateral ground-glass opacity with subpleural/lobular sparing, sometimes consolidation or upper-lobe centrilobular nodules.

448 Williams-Campbell Syndrome. Inspiratory/expiratory CT shows peripheral cystic bronchiectasis from deficient cartilaginous support in fourth- to sixth-order bronchi, with dynamic ballooning/collapse.

Rheumatology

First priority 1 topic

449 Secondary hypertrophic osteoarthropathy. Digital clubbing with symmetric long bone periostosis and parallel cortical scintigraphic uptake.

Next priority 9 topics

450 Camptodactyly arthropathy coxa vara pericarditis syndrome. Camptodactyly, noninflammatory joint swelling, and coxa vara with short femoral necks.

451 Dialysis related beta 2 microglobulin amyloidosis. Periarticular bone cysts and destructive spondyloarthropathy after long term dialysis.

452 Fibroblastic rheumatism. Periarticular cutaneous nodules with rapidly developing flexion contractures.

453 Multicentric osteolysis nodulosis and arthropathy. Carpal and tarsal osteolysis with palmoplantar nodules and contractures.

454 Pachydermodactyly. Painless lateral proximal interphalangeal soft tissue thickening without erosions.

455 Palmar fasciitis and polyarthritis syndrome. Rapidly progressive woody palmar thickening with flexion contractures and polyarthritis.

456 Singleton Merten syndrome. Early aortic or valve calcification, abnormal dentition, and acro osteolysis.

457 Stiff skin syndrome. Indurated bound down skin with hypertrichosis and joint contractures.

458 Voriconazole induced periostitis. Irregular bulky periosteal new bone, often involving ribs, clavicles, or scapulae.

Urology

First priority 8 topics

459 Bladder calculus. Plain film may show a spiculated “jackstone” in bladder; confirm at cystoscopy.

460 Bladder rupture. Retrograde CT cystogram shows extraperitoneal perivesical/Retzius leak or intraperitoneal contrast outlining bowel.

461 Ectopic ureter. MR urography depicts ureter inserting distal to sphincter; continuous incontinence despite normal voiding in a girl.

462 Prostatic abscess. Fever/dysuria with rim-enhancing intraprostatic fluid collection on TRUS, CT, or MRI.

463 Renal trauma. Multiphase contrast CT grades laceration/vascular injury; delayed phase demonstrates urinary extravasation and perirenal hematoma.

464 Traumatic urethral injury. Retrograde urethrogram shows contrast extravasation at membranous urethra after pelvic fracture or bulbar urethra after straddle injury.

465 Upper-tract urothelial carcinoma. CT urography shows enhancing filling defect or urothelial thickening in renal pelvis/ureter; ureteroscopy shows papillary tumor.

466 Ureterocele. Voiding/cystographic study shows intravesical cobra-head ureterocele; duplex obstruction may produce drooping-lily sign.

Next priority 27 topics

467 Acute idiopathic scrotal edema. Acute scrotal wall edema/hyperemia with normal testes and epididymides; transverse Doppler “fountain” pattern.

468 Anterior urethral valves. VCUG shows dilated anterior urethra proximal to valve, sometimes with anterior urethral diverticulum.

469 Concealed (buried) penis. Normal corporal length concealed by suprapubic/scrotal skin, visible as a buried shaft; stretched length is normal.

470 Congenital bladder diverticulum (Hutch). VCUG demonstrates paraureteral bladder outpouching near ureteral orifice, often associated with reflux.

471 Congenital megalourethra. Penis balloons during voiding; VCUG shows markedly dilated urethra without distal valve.

472 Congenital penile curvature. Persistent penile bend with orthotopic meatus, typically evident with erection; curvature documented by erection photograph.

473 Congenital renal arteriovenous malformation. Gross hematuria with serpiginous/corkscrew suburothelial vessels; angiography shows cirsoid AVM nidus.

474 Cowper syringocele. Urethrogram shows ventral bulbar channel/filling defect; MRI/US localizes dilated Cowper duct.

475 Encrusted cystitis and pyelitis. CT shows thin/mural calcified plaques coating bladder or collecting-system urothelium, often with alkaline urine.

476 Fibrous pseudotumor of the tunica vaginalis. Scrotal US shows multiple solid extratesticular tunical nodules, often T2-dark on MRI, with testis spared.

477 Idiopathic scrotal fat necrosis. Prepubertal boy with focal hyperechoic scrotal fat adjacent to normal testes, sometimes following cold exposure.

478 Median raphe cyst. Small midline ventral penile/perineal cyst along median raphe, apparent at birth or later.

479 Megacystis-megaureter syndrome. Imaging shows markedly enlarged smooth bladder with bilateral megaureters/reflux and no urethral obstruction.

480 Neonatal paraurethral (Skene) cyst. Newborn interlabial cyst displaces urethral meatus off midline; US shows paraurethral cyst.

481 Nonvenereal sclerosing lymphangitis. Firm mobile serpiginous cord encircles or lies just proximal to coronal sulcus, usually without dorsal vein thrombosis.

482 Pelvic lipomatosis. CT shows abundant pelvic fat compressing bladder into an inverted-teardrop shape, often with upper-tract obstruction.

483 Penile Mondor disease. Painful palpable dorsal penile cord; Doppler shows thrombosed superficial dorsal vein with absent flow.

484 Penoscrotal transposition. Scrotum lies cranial to penis, partially or completely; photograph defines degree of transposition.

485 Persistent Mullerian duct syndrome. Phenotypic male with uterus/tubes on pelvic imaging, often discovered during hernia/cryptorchidism surgery.

486 Pyeloureteritis cystica. Urography shows multiple small smooth round filling defects in renal pelvis/ureter from mucosal cysts.

487 Retrocaval ureter. Right ureter courses behind IVC with reverse-J/fishhook or S-shaped course on urography.

488 Sclerosing lipogranuloma (penile paraffinoma). Deforming penile nodules, induration, ulcers, or draining sinuses after injection of paraffin/oil.

489 Segmental testicular infarction. Acute pain with wedge-shaped avascular intratesticular segment and preserved flow in remainder.

490 Testicular epidermoid cyst. Small intratesticular avascular mass with concentric onion-ring/lamellated ultrasound echoes.

491 Urethral duplication. Two urethral openings/channels demonstrated on exam and VCUG/retrograde urethrogram.

492 Webbed penis. Scrotal skin extends onto ventral shaft and erases penoscrotal angle, often apparent with erection/traction.

493 Zinner syndrome. MRI/CT triad: unilateral renal agenesis, ipsilateral seminal-vesicle cyst, and ejaculatory-duct obstruction.

Content for existing pages

These proposals add useful content to current topics. They are excluded from the new-topic count. Broaden a title where needed and keep different diseases or etiologies clearly labeled.

Acromegaly. Add X-linked acrogigantism.

Acute Aortic Dissection. Add Aortic Intramural Hematoma.

Alpha-Thalassemia (Hemoglobin H Disease). Add Hemoglobin Bart's hydrops fetalis.

Branchial Cleft Cyst and Sinus. Add Pyriform sinus fistula (third or fourth branchial anomaly).

Cholesteatoma. Add Congenital cholesteatoma.

Congenital Adrenal Hyperplasia. Add Lipoid congenital adrenal hyperplasia; 17α-hydroxylase deficiency.

Conjunctival Lymphoma. Reorganize gastric and ocular MALT content under extranodal marginal zone lymphoma; preserve other conjunctival lymphoma types.

Cushing's Syndrome. Add Primary bilateral macronodular adrenal hyperplasia.

Dacryocystitis and Nasolacrimal Duct Obstruction. Add Congenital dacryocystocele.

Duchenne Muscular Dystrophy. Add Becker muscular dystrophy.

Encephalocele. Add Basal (intranasal) meningoencephalocele.

Exogenous Lipoid Pneumonia. Add Fire-Eater Pneumonia.

Focal Segmental Glomerulosclerosis (FSGS). Add HIV-associated nephropathy.

Gallstones and Acute Cholecystitis. Add Porcelain Gallbladder; Emphysematous Cholecystitis; Xanthogranulomatous Cholecystitis.

Hypertrophic Obstructive Cardiomyopathy. Add Apical hypertrophic cardiomyopathy phenotype.

Intravascular large B-cell lymphoma. Expand neurologic MRI coverage; the disease itself is already listed.

Isthmic Spondylolisthesis. Add Degenerative spondylolisthesis.

Lumbar Disc Herniation. Add Cauda equina syndrome.

Lumbar Spinal Stenosis. Add Cauda equina syndrome.

Osteoarthritis (Hand, Knee and Hip). Add Erosive hand osteoarthritis; Hallux rigidus.

Osteosarcoma. Add Periosteal osteosarcoma.

Rickets and Osteomalacia. Add Vitamin D-dependent rickets type 1.

Sickle Cell Disease. Add Renal papillary necrosis.

Subacute Combined Degeneration of the Spinal Cord. Add Copper deficiency myeloneuropathy; Nitrous oxide induced myeloneuropathy.

Tetralogy of Fallot. Add Absent Pulmonary Valve Syndrome.

Thoracic Empyema Necessitans. Add Pleural Empyema.

Tracheobronchial Amyloidosis. Add Nodular Pulmonary Amyloidosis.

Tympanic Membrane Perforation. Add Chronic suppurative otitis media.

Urachal carcinoma. Broaden title to Urachal Anomalies and Carcinoma before adding benign urachal cysts as a clearly separate section.

Urticaria (Acute and Chronic Spontaneous). Add Chronic inducible urticaria.

Vesicoureteral Reflux. Add Reflux nephropathy.

Required catalog corrections

- Adamantinoma has an incorrect angiosarcoma-of-bone alias. Remove that alias; the two tumors are different. Bone angiosarcoma remains deferred for independent clinical and imaging review.
- Use Hypertrophic Cardiomyopathy as the parent for obstructive and apical phenotypes. Use Dystrophinopathies for the Duchenne and Becker spectrum. Broaden Alpha-Thalassemia beyond the current HbH label before including Hb Bart's hydrops fetalis.
- For MALT lymphoma, reorganize gastric and ocular marginal-zone disease together while preserving other conjunctival lymphoma histologies. Do not relabel every conjunctival lymphoma as MALT.
- Broaden Pleural Empyema Necessitans to Pleural Empyema, and Localized Tracheobronchial Amyloidosis to Localized Pulmonary Amyloidosis, before adding their proposed related forms.
- For degenerative spondylolisthesis, broaden the current isthmic title and preserve the distinction between a pars defect and an intact pars. For dorsal-column myelopathy, distinguish copper deficiency from functional vitamin B12 inactivation by nitrous oxide.
- Use Alveolar Macrophage Pneumonia as the current ERS/ATS term, with Desquamative Interstitial Pneumonia and DIP retained for search. Treat isolated left ventricular hypertrabeculation as a morphology requiring clinical interpretation, rather than automatically labeling it a separate cardiomyopathy.

Selected clinical references

These references support selected imaging patterns, nomenclature, and consolidation decisions. They are reading references; image reuse requires its own license assessment.

1. 2025 ERS ATS statement on interstitial pneumonia classification

2. 2023 ESC guidelines for cardiomyopathies

3. 2022 ESC ERS guidelines for pulmonary hypertension

4. 2023 EAN PNS guideline on Guillain Barre syndrome

5. Clinical and radiologic features of Balo concentric sclerosis

6. Enterovirus 71 infection and neurologic complications

7. Dorsal thoracic arachnoid web and the scalpel sign

8. GeneReviews on biotin thiamine responsive basal ganglia disease

9. GeneReviews on HTRA1 disorder

10. GeneReviews on CSF1R related disorder

11. GeneReviews on LAMA2 muscular dystrophy

12. GeneReviews on multicentric osteolysis nodulosis and arthropathy

13. Imaging findings of COPA syndrome

14. Clinical features of molecularly confirmed Sanjad Sakati syndrome